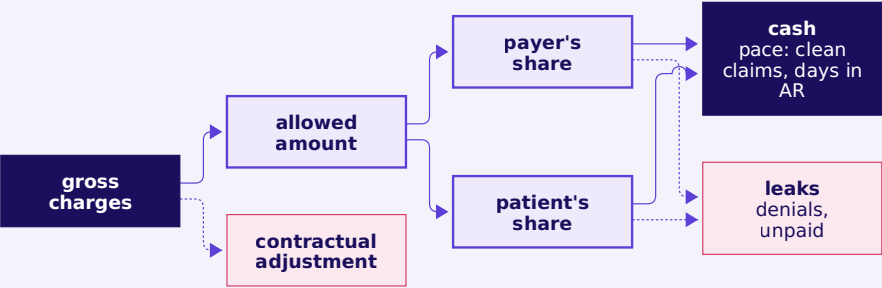


1 From a test's charge to cash



Allowed amount = payer's share + patient responsibility. Contracts take most of the list price away before anyone pays, denials and unpaid balances leak out of what is left, and the clean claim rate and days in AR set the pace of the rest.

Crux. Place every revenue-cycle number on this tree, as a charge, an allowed dollar, a leak or the pace of cash, before explaining a change in it.

2 The ten metrics, as formulas

Metric	Formula
Gross charges	Tests billed x list price
Allowed amount	Payer's share + patient's share
Contractual adjustment	Gross charges less allowed
Net revenue	Gross charges less contractual adjustments less expected uncollected
Patient responsibility	Deductible + coinsurance + copay
Denial rate	Denied at first answer / submitted
Clean claim rate	Passed every edit untouched / entered
Days in AR	Receivable / net revenue per day
Net collection rate	Payments / (gross charges less contractual adjustments)
Turnaround time	Draw to released result, median

Crux. Say the divisor, and whether it counts claims or dollars.

3 Traps that make a lab's number lie

Trap	The check
List-price rise read as growth	Track allowed dollars
Allowed amount read too early	Wait for slow payers
Denials by count against dollars	State the basis
Days in AR cut by write-offs	Read write-offs beside it
Collections over gross charges	Divide by allowed

Crux. Run each check before the number leaves the team.

4 Twenty words to say fluently

Word	What it means
Payer	Whoever pays the claim
Medicare	Federal cover, mostly 65 and over
Medicaid	State cover for low incomes
Requisition	The doctor's order for tests
Accession number	Barcode tying sample to order
Panel	Tests billed under one code
Chargemaster	The lab's own price list
Deductible	Paid by the patient before the plan pays, each plan year
CPT	The AMA's procedure codes
ICD-10-CM	Diagnosis codes: why it was ordered
NPI	A provider's ten-digit number
837, 835	The claim, and the remittance
Clearinghouse	Checks and routes claims
Rejection	Bounced before the payer saw it
Denial	The payer's refusal to pay
CARC	Why a dollar went unpaid
Prior authorisation	Payer approval before a service
Medical necessity	The service fits the diagnosis
Timely filing limit	The deadline to submit a claim
PHI	Health data that identifies a patient

5 The rules a US lab's data team works under

Rule	What the data team does
HIPAA Privacy	Treats identifiable rows as PHI
Minimum necessary	Asks only for the columns needed
De-identification	Expert-certified, or 18 identifiers out
Business associate	Works inside the agreement
Security, breach	Role-based access; notice within 60 days
Medicare necessity	Never adds a diagnosis to get paid
Timely filing	Claims nearest the limit first; Medicare one year
Offshore	Follows the agreement and contracts

6 Where \$100 of gross charges goes

Line	Left, \$
Gross charges	100
Less contractual adjustments	40 allowed
Less denials upheld, balances unpaid	37 net revenue
Less the cost of the tests	12 gross profit
Less billing, sales, technology	5.50 operating income

Crux. On these illustrative numbers a \$1 leak from the \$40 allowed is 2.5 percent of it and 18 percent of the lab's operating income.